

Poshan Saathi: SHG-led Kitchen Gardens, Nutrition Counselling and ICDS Convergence for Mothers and Young Children in Murhu, Khunti

A 24-month proposal from EcoKheti Foundation to improve diets, maternal and young-child feeding practices and the uptake of ICDS and health services among about 1,200 women from SHG member households in 20 villages of Murhu block, Khunti district, Jharkhand. The budget is Rs 1,39,89,608, against an indicative ceiling of Rs 1,40,00,000.

Location: Murhu Block, Khunti District, Jharkhand

Duration: 24 months (8 quarters)

Budget: Rs 1,39,89,608 against an indicative ceiling of Rs 1.4 Crore (Rs 1,40,00,000) over 2 years. The ceiling is still to be confirmed.

Submitted by: EcoKheti Foundation

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Problem Statement

This chapter explains why mothers and young children in Murhu block remain undernourished. It also explains why the situation will not correct itself without a deliberate intervention. Undernutrition in Murhu has no single cause. Poverty, remoteness, low female literacy, limited year-round access to diverse food, and incomplete use of public entitlements all feed it, and each makes the others worse. The sections below examine each dimension in turn. They then draw these threads into a single causal account that the rest of the proposal responds to.

Geographic and administrative context

Khunti district had a population of 5,31,885 at the 2011 Census. Of this, 91.55% was rural, 73.25% was Scheduled Tribe and 4.52% was Scheduled Caste, and forest covers about 40% of the district area ^{[S1][S4]}. Khunti is therefore overwhelmingly rural, overwhelmingly tribal and heavily forested. Murhu block goes further on each count. Its population of 85,486 was entirely rural, 79.19% was Scheduled Tribe, and 13,907 children were aged 0–6 at the time of the Census ^[S2]. These block figures come from a secondary compilation and will be verified against the District Census Handbook. The number of villages and households in the block is also a gap that the DCHB will fill.

An entirely rural block with a large forest share usually has dispersed settlements and long distances to services. The Anganwadi centre, the sub-centre and the Community Health Centre are the points through which the State delivers nutrition and health care. When a mother has to travel far to reach them, each visit costs her time, wages and effort. The next nearest Malnutrition Treatment Centre for Murhu has not been identified, and this is itself a gap. Distance is likely to weaken the referral chain for the sickest children. Access and connectivity will be assessed before villages are selected.

Social and community structure

The Munda make up 83.66% of Khunti's Scheduled Tribe population and the Oraon 8.52% ^[S3]. This figure comes from a secondary compilation and still needs verification, and the tribal composition specific to Murhu is not yet known.

This matters for design as well as description. In communities with strong collective traditions, change spreads most readily through groups the community already recognises. Messages delivered by outsiders tend to spread far less. The SHGs and Village Organisations already present in Murhu are such institutions. Working through them respects community ownership, and it gives nutrition messages a trusted route into households. A proposal that ignored these structures and built parallel groups would add cost and weaken local legitimacy.

Economic and livelihoods dimension

The Below Poverty Line share in Khunti was 35.45% in 2011-12 ^[S5]. The original government source is still being located. On this figure, roughly a third of households in the district lived in poverty. Such households have little cash to buy pulses, eggs, fruit or other diverse foods when their own fields do not produce them.

Block-level data on livelihoods, landholding, migration and the Public Distribution System are not available. This is a real limitation, because the pattern of household food supply across the year determines when diets thin out. The baseline will therefore record household food security, land and water access, and migration. In a poor rural economy, the diet tends to narrow to staples in lean periods. These are the months when a pregnant woman or a weaning child most needs variety. Income alone will not close this gap quickly. A household that can grow some of its own diverse food, however, is less exposed to both seasonal scarcity and cash shortage.

Gender and education: the literacy gap and its consequences

Female literacy in Khunti is 53.69%, compared with 74.08% for males ^[S1]. Murhu shows almost the same pattern, at 53.66% for women and 73.15% for men ^[S2]. At state level, 32% of women aged 15–49 in Jharkhand have never attended school ^[S6].

A gap of about twenty percentage points means that close to half the women who manage household food and child feeding cannot rely on written material. Printed leaflets, text-heavy Mother and Child Protection cards and written scheme instructions will largely fail to reach them. The gap also affects entitlements. Enrolling in PMMVY, seeding a bank account or keeping an MCP card up to date all involve documents that low-literacy women find hard to handle alone.

Low literacy therefore feeds undernutrition along two routes, knowledge of feeding practices and access to benefits. Any serious response has to rely on oral, pictorial and face-to-face counselling. It should be delivered by women from the same community, who share the listener's language and circumstances. The literacy gap also points to a wider imbalance in women's voice and control. Building women's agency through their own SHGs is therefore a nutrition strategy as well as an equity goal.

Health and nutrition burden

Undernutrition in Jharkhand is severe. According to NFHS-5 (2019-21), 40% of children under five in the state are stunted, 22% are wasted, 9% are severely wasted and 39% are underweight ^{[S6][S10]}. The national figures are 35.5% stunted, 19.3% wasted and 32.1% underweight, so Jharkhand is worse on every measure ^{[S6][S10]}. Anaemia deepens the picture. Some 67% of children in Jharkhand are anaemic, and anaemia among women has not fallen since NFHS-4 ^{[S7][S6]}.

These figures describe more than a state average. All 24 districts of Jharkhand, Khunti included, reach public health concern on WHO thresholds for stunting, wasting, severe wasting and anaemia ^{[S8][S9]}. Under NFHS-4, Khunti was among the districts with the highest

shares of underweight and severely wasted children ^{[S8][S9]}. Khunti's NFHS-5 district values will be taken from the IIPS district fact sheet. Values for the project area will be established by the baseline survey.

Each of these indicators has a human consequence. Stunting reflects chronic deprivation, usually beginning in pregnancy and the first two years, and it is largely irreversible after that window. Wasting and severe wasting sharply raise the risk of death from common illness. Maternal anaemia that has not improved across two survey rounds implies that many infants start life already short of iron. When anaemic mothers give birth to low-reserve infants, and those infants are then weaned onto iron-poor diets, the cycle passes from one generation to the next.

Feeding practices in the first 1,000 days

Only 22% of children aged 6–23 months in Jharkhand ate iron-rich foods on the previous day ^[S6]. Put the other way, nearly four in five children at the peak weaning age are missing iron-rich foods. Given that 67% of children are anaemic, this is a direct and predictable link.

Minimum dietary diversity, minimum adequate diet and exclusive breastfeeding rates for Khunti and for the target households are not yet known, and the baseline will establish them. The state picture is enough to show that complementary feeding is a central weak point. It is where limited food access and limited knowledge meet. A mother may have greens, pulses or eggs available only part of the year. She may also not know that her young child needs them in particular quantities and textures. Either failure alone narrows the child's diet. Together they almost guarantee a narrow one.

Access to government schemes and entitlements

The public system offers substantial support on paper. Poshan 2.0 entitles families to supplementary nutrition through Take Home Ration, nutrition and health education, immunisation, health check-ups and referral ^[S22]. How far families in Murhu actually receive these services has not been documented. This includes the number of Anganwadi centres, the regularity of THR, growth monitoring coverage, the functioning of Village Health, Sanitation and Nutrition Days, and uptake of antenatal care and PMMVY. These gaps will be established at baseline and through a mapping of Anganwadi centres and health facilities.

The state also runs targeted programmes, including Didi Badi Yojana, POSHAN PEHL and SAMAR. How far they reach into Murhu has not been mapped ^{[S25][S26]}. This matters in two ways. Where entitlements go unclaimed, families lose food, cash and care they are already owed. Where programmes overlap without coordination, resources are duplicated while other households are missed.

Unclaimed entitlements often come down to practical barriers such as missing documents, meeting times that clash with farm work, low awareness of what is due, or supply gaps at the centre itself. This field experience is consistent with the literacy gap described above. Barriers

on the demand side and on the supply side call for different responses, and this project will track them separately.

The weak continuum of care for severely malnourished children

Jharkhand runs 88 Malnutrition Treatment Centres under the National Health Mission, and Anganwadi Workers detect severe acute malnutrition in the community ^[S12]. Yet one study found that at the end of the community phase of the MTC programme, 52% of children were still severely wasted and 83% were underweight ^[S12]. The study points to weak follow-up after discharge.

The implication is serious. A child may be identified, admitted and treated, and then return to the same household conditions that caused the wasting, with no structured support at home. In that case, much of the value of facility treatment is lost. The broken link lies in the community, between discharge and full recovery. Neither the MTC nor an overstretched Anganwadi Worker can close it alone. A trained community cadre working alongside the Anganwadi Worker can.

Agro-ecological and environmental dimension

Forest covers about 40% of Khunti district ^{[S1][S4]}. Forests are a potential source of indigenous greens and other foods, and local food mapping will document what households actually use. Field evidence on nutrition gardens in Jharkhand is limited, but it does note land and water constraints ^[S19]. Agro-climatic data for the project area are a gap.

Homestead land and water are therefore the binding constraints for any food production approach here. A garden that depends on reliable irrigation or large plots will fail in the dry months, which is precisely when it is most needed. Dry spells, an erratic monsoon and livestock damage are real threats. Any garden model must be climate-resilient by design. That means drought-tolerant indigenous crops and millets, greywater use, seed saving and plots sized to the land and water available, with crop choices confirmed with KVK Khunti.

Migration and its social cost

Migration data for Murhu are not available, and this analysis does not assume their scale. Where a household member migrates for work, the pattern usually shifts farm and domestic labour onto the women who remain. That leaves less time for garden upkeep, Anganwadi visits and careful child feeding. The baseline will record migration so that the project can adjust the timing and intensity of its support where it is a significant factor. Field data will sharpen this point considerably.

Why the problem persists and will not resolve on its own

Taken together, these dimensions form a layered causal structure ^{[S25][S26]}.

- **Immediate causes.** Diets in pregnancy and the first 1,000 days are poorly diversified, and feeding and care practices are suboptimal.

- **Underlying causes.** Households lack year-round access to diverse foods. Awareness is low in households where many women cannot read. Families take up ICDS and health entitlements only partly.
- **Structural causes.** Poverty, remoteness and the literacy gap between women and men sit beneath all of the above.

These layers reinforce each other. Poverty limits food purchase. Limited land and water limit food production. Low literacy limits both knowledge and access to documents. Remoteness weakens every service. Weak follow-up allows treated children to relapse.

State programmes exist, but their reach into Murhu is unmapped. Until households, community institutions and frontline services are deliberately connected, entitlements will keep going unclaimed while children continue to miss the diets they need.

No single intervention can break this cycle. Food alone does not change feeding practice, and counselling alone does not create food where there is none. Neither will work if families cannot reach the services they are owed. A response must act on food access, behaviour and service uptake together, through institutions that women already trust. The next chapter sets out the change this proposal seeks to achieve on that basis.

Project Objective

The problem analysis identified three connected underlying causes of maternal and child undernutrition in Murhu. Households lack year-round access to diverse foods. Awareness of good feeding practice is low in households with low literacy. Families take up ICDS and health entitlements only partly. The objective of this project responds to each of these directly.

Objective: Within 24 months, improve dietary diversity and maternal and young-child feeding and care practices, and raise the uptake of ICDS and health services, among about 1,200 women from SHG member households with pregnant or lactating women and children under five in 20 villages of Murhu block, Khunti. All delivery will run through existing SHGs and government platforms.

Who the project serves. The primary participants are about 1,200 women from SHG member households that include a pregnant or lactating woman, a child under five, or both. They live in 20 villages of Murhu block. These villages will be selected in the first quarter for existing SHG presence and Anganwadi centre catchments. They will also be checked for overlap with JSLPS Didi Badi and FNHW coverage. The baseline will establish how many of these women are currently pregnant or lactating and how many children under five live in their households. These figures will be triangulated with Anganwadi registers.

What will change. Success will be measured in three areas, which follow directly from the causal analysis:

- households eat more diverse diets throughout the year;
- mothers and caregivers adopt recommended maternal nutrition and infant and young child feeding practices;
- families take up their ICDS and health entitlements more fully, and at-risk children are identified, referred and followed up.

What the objective does not claim. The objective deliberately targets diets, practices and service uptake as the primary results. Evidence from comparable interventions shows that these change within a project cycle, while nutritional status moves more slowly. Improvement in children's anthropometry is therefore treated as a contribution at the impact level and is not attributed to the project. This is a realistic target, and a 24-month project can be held accountable for it.

Timeframe. The project runs for 24 months, over eight quarters. The strategy chapter that follows explains how this objective will be achieved.

Operational Strategy

The objective commits the project to working on food access, behaviour and service uptake together. This chapter explains the approach chosen to do so, why it suits Murhu, and why the evidence and EcoKheti Foundation's own experience support it.

One platform, three pathways

The core design choice is to combine three pathways through a single platform:

- kitchen gardens improve access to food;
- counselling changes behaviour;
- convergence with ICDS and health services raises the use of public entitlements.

The existing SHGs and Village Organisations will deliver all three. No parallel groups will be created.

This choice follows directly from the problem analysis. Undernutrition in Murhu comes from causes that operate together, so a single-pathway intervention would leave the other causes untouched. A household given a garden but no counselling may grow greens and still not feed them to a weaning child. A household given counselling but no food may understand the advice and be unable to act on it. Either household may still miss THR, antenatal care or PMMVY for lack of a document. The three pathways are intended to reinforce one another inside the same households.

The choice of platform matters as much as the choice of pathways. SHGs already meet regularly, are led by women and are recognised by the community. Working through them respects EcoKheti Foundation's values of community ownership, women's agency and convergence with public systems. It also means that at the end of the project the capabilities built will belong to institutions that continue to exist.

Why women's groups can drive behaviour change: the evidence

The counselling model draws on evidence about women's groups in eastern India. This is external evidence of what is possible, not a finding from Murhu. The Ekjut participatory learning and action trial in tribal Jharkhand and Odisha worked through 244 women's groups with local women facilitators. It cut neonatal mortality by 45% in its last two years ^{[S13][S14][S15]}. The same model has since been delivered through ASHAs, which supports embedding it in government platforms ^{[S13][S14][S15]}.

Two lessons shape this project. First, groups led by local women can change health behaviour in tribal communities much like those in Khunti. Second, the model does not need to depend on an NGO indefinitely, because it can live within public systems. For this reason, the project's counselling cadre is drawn from SHG members, and its routines are tied to Anganwadi and VHSND calendars.

Why gardens are paired with counselling

The UPAVAN trial in rural Odisha is also external evidence. It found that SHG-based nutrition-sensitive agriculture combined with nutrition behaviour change raised minimum dietary diversity among children and mothers compared with controls. It also found that participation mattered ^{[S16][S17]}. For this reason, kitchen gardens in this project are always paired with counselling from the Community Nutrition Resource Persons (CNRPs). Household tracking follows participation, so that the project knows which families are engaging and can reach those who are not.

The UPAVAN protocol also notes that participatory learning and agriculture interventions change knowledge and dietary diversity strongly, but change nutritional status only a little ^[S18]. This finding is the basis for the realistic targeting set out in the objective. The primary outcomes are diets, practices and service uptake, and anthropometry is tracked at impact level as a contribution only ^[S18]. It protects the project from promising what 24 months cannot deliver. It also directs attention to the intermediate changes that make later gains in nutritional status possible.

A garden model built for Murhu's constraints

The garden design builds on EcoKheti Foundation's climate-resilient agriculture model, which reached 70% adoption among 800 farmers in Chhattisgarh. It uses millets, indigenous greens, fruit saplings, greywater and seed saving. Field evidence from Jharkhand is limited, but it notes land and water constraints ^[S19]. Plots are therefore sized to the homestead land and water each household has. Agro-climatic data for the block are a gap, so crop lists will be confirmed with KVK Khunti and through community food mapping.

The model responds to the risks identified in the problem analysis:

- **Dry spells and erratic rainfall.** The project will use drought-tolerant indigenous crops, mulching and staggered sowing.
- **Livestock damage.** Gardens will use live fencing and local materials.
- **Recurring seed costs.** Community seed banks run by Village Organisations will supply seed from the second year onwards.

Convergence as a strategy, not an afterthought

The project is designed to add value to public programmes and to avoid duplicating them. Each convergence route is specified below.

- **Didi Badi Yojana (JSLPS Poshan Vatika).** The project will map existing coverage with JSLPS Murhu before starting. Where households are already covered, it will add counselling and service linkage, and it will not supply duplicate kits ^{[S25][S26]}.
- **Poshan Vatika at Anganwadi centres.** Village demonstration plots will be placed at or near Anganwadi centres or on Gram Panchayat land, in line with Government of India guidance ^[S23]. This makes the Anganwadi centre a visible centre of nutrition learning.

- **Poshan 2.0 and Saksham Anganwadi.** The project will help families take up the six Anganwadi services. Its community growth tracking supplements growth monitoring and does not replace it, and its data are reconciled with Anganwadi Worker registers ^[S22].
- **PMMVY and JSY.** CNRPs will help pregnant women prepare their documents, including Aadhaar, bank seeding and the MCP card, before Anganwadi Workers and ASHAs register them. PMMVY pays Rs 5,000 in two instalments for the first child, and Rs 6,000 for a second child who is a girl. JSY is paid after institutional delivery ^[S24]. These benefits reach families through convergence and are not a project cost. The low female literacy documented earlier is exactly why this documentation support matters.
- **MGNREGA.** Garden fencing, water structures and labour will be sought through MGNREGA works. Scheduled Tribe households in forest areas are entitled to 50 extra days of work on top of the standard 100 ^[S27]. The project budget covers fencing and water support only for the demonstration plots.
- **NHM and MTC.** The project strengthens the link from Anganwadi SAM detection to the MTC. It adds community follow-up after discharge, in response to the poor recovery after MTC discharge seen in Jharkhand ^{[S12][S20][S25]}. The status of SAMAR and POSHAN PEHL in Khunti will be checked, and the project will align with them.

Convergence also depends on supply. If THR is irregular, if Anganwadi Worker or ASHA posts are vacant, or if VHSNDs are not held, raising demand alone will not raise uptake. The project will map service gaps in the first quarter. It will raise supply issues at quarterly block and half-yearly district convergence meetings, and it will track service availability separately from demand-side uptake.

The project has no formal working relationship yet with the ICDS project office, the block health team or JSLPS. Letters of support or an MoU will be sought from the Murhu CDPO, the Medical Officer in charge of the CHC and the JSLPS Block Project Manager during the inception quarter. EcoKheti Foundation will build on its existing SRLM relationship in Khunti.

Why EcoKheti Foundation is placed to deliver this

EcoKheti Foundation is a Section 8 non-profit company, registered in 2016 and working in Jharkhand and Chhattisgarh. It has 8 years of experience in community-based livelihoods, 6 in maternal and child nutrition, and 5 in climate-resilient livelihoods. Its annual revenue is Rs 2.4 Crore, and it has 9 permanent staff and a 5-member board.

The project brings together three strands of the organisation's track record:

- an SRLM-funded SHG-led livelihoods and nutrition project with 1,200 women in Khunti and Gumla, which reported an income increase of at least 45% and improved dietary diversity in 60% of households;
- a CSR-funded climate-resilient agriculture project with 800 farmers in Chhattisgarh, where 70% adopted climate-adaptive practices;

- the strengthening of 3 FPOs with 900 members in Jharkhand, which roughly doubled their aggregated sales.

Whether the target villages overlap with the earlier Khunti project area will be confirmed during inception. The organisation's familiarity with SHG institutions in the district is a direct asset in either case.

The strategy therefore sets out a clear logic: three pathways through one trusted platform, grounded in regional evidence, and tied into public systems from the start. The next chapter traces how this strategy produces results at each level.

Activities

The previous chapter set out what the project will achieve. This chapter describes how it will be achieved. The twelve activities are grouped by the part of the Theory of Change they serve. For each group, the chapter explains the mechanism that turns the activity into its intended result. The sequence matters. Inception and baseline come first, so that every later activity is built on actual knowledge of Murhu and not on assumptions.

Laying the foundation: inception, baseline and village planning (A1–A3)

A1 Inception and mobilisation (Q1). In the first quarter, the project will select and name the 20 villages. It will map the SHGs, Village Organisations, JSLPS coverage, Anganwadi centres, ASHAs, VHSND functioning and the nearest MTC. SHGs and Village Organisations will be oriented, and pregnant and lactating women and children under five will be identified. The project will also seek support letters or MoUs with the CDPO, the block health team and JSLPS.

This is the activity that makes convergence real. Mapping JSLPS Didi Badi and FNHW coverage with the Murhu Block Mission Management Unit lets the project choose villages and households that complement what already exists. Where gardens are already in place, the kit budget will be redirected to counselling and convergence. Mapping service gaps early lets supply and demand be tracked separately from the start. Assessing access and connectivity before village selection protects field presence and referral routes.

A2 Baseline survey (Q1–Q2). A sample survey of about 500 households, carried out by a regional agency, will measure:

- household dietary diversity;
- maternal and child minimum dietary diversity;
- IYCF practices;
- service uptake, including ANC, THR, immunisation and PMMVY;
- food security;
- anthropometry within the sample.

It will also fill the gaps identified in the problem analysis, including land and water access and migration. Every "to be established" figure in the logframe will be established here, so the endline in Q8 can measure real change.

A3 Village nutrition action plans (Q2 and Q5). In each of the 20 villages, SHGs will develop and own a nutrition action plan through a participatory learning cycle. This applies the participatory learning and action approach that proved effective among women's groups in tribal Jharkhand and Odisha ^{[S13][S14][S15]}. When women analyse their own nutrition problems and decide their own actions, the project's activities become their plan and not an outside agenda. The plans will be revisited in Q5, so that the second year reflects what the first year taught.

Growing diverse food: gardens, demonstration plots and seed banks (A4–A5), delivering Outcome 1

A4 Nutri-kitchen gardens (Q2–Q6). The project will run 40 one-day household training batches of 30 participants each, covering about 1,200 households. Each household will receive seed kits for two seasons, including vegetables, greens, millets and pulses, and one set of five fruit saplings, such as moringa, papaya and lemon. The Kitchen Garden Specialist and the CNRPs will provide seasonal follow-up at the demonstration plots.

The mechanism is simple. Training gives households the skills to establish and keep a garden. Seed provides the initial inputs. Fruit saplings add perennial sources of micronutrients that keep producing after the project ends. Seasonal follow-up addresses the problems that cause gardens to fail, such as pests, water stress and livestock. The garden design uses drought-tolerant indigenous crops, mulching, staggered sowing and greywater, and each plot is sized to the household's land and water. Crop lists will be confirmed with KVK Khunti and through community food mapping. Household fencing and water structures will be sought through MGNREGA ^[S27].

A5 Demonstration plots and seed banks (Q2–Q8). The project will set up 20 demonstration plots at or near Anganwadi centres or on Gram Panchayat land ^[S23]. Each will have fencing and low-cost water management, such as live or bamboo fencing, a greywater channel and drum storage. Alongside them, 20 community seed banks will be run by Village Organisations. Seed for the second year will come from these banks.

The demonstration plots let women see and practise techniques before applying them at home. Placing them near the Anganwadi centre links garden learning to the place where mothers already bring their children. The seed banks are what turn a one-time distribution into a lasting system. After Year 1, seed circulates within the community under Village Organisation management and no longer depends on project funds.

Changing practice: the CNRP cadre and counselling (A6–A7), delivering Outcome 2

A6 Community Nutrition Resource Persons (Q1–Q7). The project will select 40 CNRPs, two per village, through the SHGs, with endorsement from the Village Organisations. They will receive six residential training modules, delivered as three batches of two modules each, and six non-residential refresher days each. Each CNRP will be equipped with a pictorial counselling kit in the local language. Laminated flipbooks will also go to the 20 Anganwadi centres, so that the CNRP and the Anganwadi Worker use the same messages.

Selection by the SHGs gives each CNRP legitimacy among her peers. Two CNRPs per village provide redundancy against drop-out, which is a real risk because the honorarium is modest and domestic workloads are heavy. The honorarium will be confirmed against the JSLPS CRP norm, and the federation will give CNRPs formal recognition. Pictorial materials respond directly to the literacy gap. In Murhu, 53.66% of women and 73.15% of men are literate ^[S2], and 32% of women aged 15–49 in the state have never attended school ^[S6].

A7 Counselling delivery (Q2–Q8). CNRPs will:

- hold monthly group sessions within SHG meetings;
- run 440 cooking demonstrations with local foods, one per village per month for 22 months;
- make targeted home visits during pregnancy, lactation and the first 1,000 days;
- give each household pictorial take-home cards.

Each channel does different work. Group sessions shift norms within the group, so that good feeding practice becomes the expected practice. Cooking demonstrations teach women how to prepare iron-rich and diverse complementary foods from what their gardens and forests provide. This addresses directly the state finding that only 22% of children aged 6–23 months ate iron-rich foods ^[S6].

Home visits are timed to the moments when decisions are made, such as late pregnancy, the first days after birth and the start of complementary feeding. Take-home cards keep the message visible to other family members, including mothers-in-law and husbands, who influence what a child eats. Together, the four channels connect food availability under Outcome 1 to what children and mothers actually eat under Outcome 2.

Connecting families to services: convergence, referral and frontline capacity (A8–A10), delivering Outcome 3

A8 ICDS and health convergence (Q2–Q8). SHG meetings will be aligned with Anganwadi and VHSND calendars. CNRPs will help families prepare for THR, ANC and PNC, and immunisation. They will also help women get their PMMVY and JSY documents ready, including Aadhaar, bank seeding and the MCP card, before Anganwadi Workers and ASHAs register them ^[S24]. SHG members will be supported to take part in VHSND.

Each step removes a specific barrier. Aligned calendars remove timing conflicts. Documentation support removes the paperwork barrier that low literacy creates. SHG participation in VHSND strengthens the platform itself. The project treats Poshan 2.0 services as entitlements that families should be receiving ^[S22]. PMMVY and JSY benefits reach families as convergence and are not a project cost.

A9 Early identification and referral (Q3–Q8). One supplementary growth-monitoring kit per village, containing a scale, MUAC tapes and an infantometer, will be used alongside the Anganwadi Worker. The data will be reconciled with Anganwadi registers. At-risk mothers and children will be tracked, and children with SAM will be referred to the MTC. After discharge, CNRPs will follow up each child together with the Anganwadi Worker.

This activity responds to the evidence that 52% of children in Jharkhand's MTC programme were still severely wasted at the end of the community phase ^[S12]. The community-level follow-up, rather than any change at the facility, is what strengthens the chain. The CNRP returns to the household after discharge and supports feeding with garden and local foods. She also keeps the family connected to the Anganwadi Worker and the MTC.

A referral contingency is budgeted to cover transport or an escort where government transport is not available. It is planned at 60 cases, which is a planning figure and not a prevalence estimate. Referrals will use JSY and NHM transport first.

A10 Frontline capacity building (Q1–Q8). The project will hold eight joint training batches of about 30 Anganwadi Workers, ASHAs and SHG leaders each, over four rounds. It will run two in-state exposure visits for 60 leaders and CNRPs, and it will hold eight quarterly block convergence meetings and four half-yearly district convergence meetings.

Joint training matters because the Anganwadi Worker, the ASHA and the SHG leader often work in the same village without a shared routine. Training them together creates common expectations and shared messages. Exposure visits let peers learn from peers.

The convergence meetings serve two purposes. They are where supply problems, such as irregular THR, vacant posts or VHSNDs not being held, are raised with ICDS and health officials. They are also the routine that the project will embed in block processes before it ends. District meetings bring in the District Social Welfare Officer, the Civil Surgeon and JSLPS.

Learning and handover (A11–A12), delivering Output 4

A11 Monitoring, evaluation and learning (Q2–Q8). A mobile household tracking system will run on 24 devices, held by the four field facilitators and 20 lead CNRPs, using an open-source data collection tool. It will follow mothers, children and gardens. Quarterly reviews will be held with SHG federations and block officials, the endline survey will take place in Q8, and the project will produce case studies and learning briefs.

Tracking is a management tool as well as a reporting tool. It shows which households have missed home visits, which gardens have failed and which children are overdue for follow-up. Because the UPAVAN evidence found that participation mattered ^{[S16][S17]}, the project monitors participation household by household. Quarterly reviews with federations and officials make these data a shared basis for decisions, and not an NGO record alone.

A12 Sustainability and handover (Q7–Q8). In the final two quarters, seed banks will be formally transferred to Village Organisations. CNRPs will be linked to SHG federations and, where possible, to JSLPS cadres, and convergence routines will be recorded in the minutes of block meetings. The next chapter describes this activity in full.

Staffing and timing

A Project Coordinator will lead the team, working alongside a Nutrition and Health Convergence Officer, a half-time Kitchen Garden Specialist, a half-time MEL and Data Officer, and four field facilitators. Each field facilitator will be based in a cluster of five villages and will supervise the CNRPs there.

The timeline follows a logical sequence. Q1 is for mapping, agreements and the baseline. Gardens, counselling and convergence begin in Q2. Growth tracking and referral begin in Q3, once CNRPs are trained. Handover takes place in Q7 and Q8.

The budget is modular. Kits, CNRP numbers and surveys can be scaled if the donor ceiling or the target definition changes after engagement. The target definition will be confirmed during inception.

Implementation Timeline

Activity	Q1	Q2	Q3	Q4	Q5	Q6	Q7	Q8
A1 Inception, village selection, mapping, MoUs, SHG/Village Organisation orientation								
A2 Baseline survey								
A3 Village nutrition action plans								
A4 Household garden training and kits								
A5 Demonstration plots and seed banks								
A6 CNRP selection, training and refreshers								
A7 Counselling, cooking demonstrations, home visits								
A8 ICDS and health convergence, VHSND linkage, PMMVY support								
A9 Growth tracking, referral and post-MTC follow-up								
A10 Joint trainings, exposure visits, convergence meetings								
A11 Tracking system, reviews, documentation								
A11 Endline survey								
A12 Sustainability and handover								

Shaded cells indicate active periods.

Results Framework

This chapter sets out the results chain, which translates the strategy into measurable change. It describes the long-term change the project contributes to, the three outcomes it is directly accountable for, and the outputs that make those outcomes possible. For each level, it explains why that level follows logically from the one below it. The chain mirrors the causal analysis in reverse. The outputs address the underlying causes of undernutrition, the outcomes change the immediate causes, and the impact is the improved nutrition that follows over time.

Impact: a contribution to better nutrition

The impact the project works towards is a *contribution to improved nutritional status of mothers and children under five in tribal SHG households of Murhu block, Khunti*.

Indicators. The impact indicators are the percentage of children under five who are underweight and the percentage who are stunted. Both are reported as a contribution and are not attributed to the project.

Baseline and target. The baseline will be established by the project survey. The state reference points from NFHS-5 are 39% underweight and 40% stunted. The target is a declining trend against the project baseline.

This level is framed as a contribution on purpose. Nutritional status depends on disease, sanitation, household income and many other factors outside any single project's control. The evidence also shows that interventions of this type move diets and knowledge far more than anthropometry within a short period ^[S18]. Presenting anthropometry honestly protects the credibility of the whole results framework.

Outcome 1: more diverse diets year-round

Target households eat more diverse diets year-round, drawing on nutri-kitchen gardens and local foods.

Why it follows. Diverse diets depend first on diverse food being available. When about 1,200 households maintain climate-resilient gardens (OP1.1) and have seed banks and demonstration plots nearby (OP1.2), they can produce greens, vegetables, pulses, millets and fruit at home. This includes the lean periods when diets would otherwise narrow to staples. Seed banks remove the recurring cost of seed, so production can continue into the second year and beyond. Counselling under Outcome 2 ensures that the food grown is actually eaten by pregnant women and young children.

Indicators and targets.

- The percentage of pregnant and lactating women achieving Minimum Dietary Diversity for Women (MDD-W). The target is the baseline value plus 20 percentage points.

- The percentage of households consuming garden produce in each season. The target is that 70% of households consume garden produce in at least two seasons.

Means of verification. Both indicators will be measured through 24-hour dietary recall in the baseline and endline surveys and through the household tracking system.

Outcome 2: better maternal nutrition and IYCF practices

Pregnant and lactating women and caregivers adopt recommended maternal nutrition and infant and young child feeding (IYCF) and care practices.

Why it follows. The problem analysis showed that low literacy and low awareness make knowledge a binding constraint. Only 22% of children aged 6–23 months in Jharkhand ate iron-rich foods on the previous day ^[S6]. Outcome 2 rests on two outputs:

- a trained and active cadre of 40 CNRPs from the community (OP2.1);
- monthly group counselling, cooking demonstrations and home visits delivered in every village throughout the first 1,000 days (OP2.2).

Pictorial counselling in the local language, delivered by women from the same SHGs, reaches women whom printed materials cannot. Cooking demonstrations with local and garden foods turn advice into skills that can be practised. Home visits at key moments in pregnancy, lactation and weaning bring the advice to the household when a decision is actually being made. Evidence from women's groups in eastern India supports this logic ^{[S13][S14][S15]}.

Indicators and targets.

- The percentage of children aged 6–23 months achieving minimum dietary diversity. The target is the baseline value plus 15 percentage points.
- The percentage of infants under 6 months who are exclusively breastfed. The target is a significant improvement on the baseline.
- The percentage of children aged 6–23 months eating iron-rich foods. The target is a significant improvement on the baseline, measured against the NFHS-5 state reference of 22%.

Means of verification. These indicators will be measured through the baseline and endline surveys and through CNRP home-visit records.

Outcome 3: fuller uptake of entitlements and a working referral chain

Pregnant women, mothers and children under five take up ICDS and health entitlements more fully, and at-risk children are identified, referred and followed up.

Why it follows. Entitlements go unclaimed for practical reasons such as timing, documents, awareness and weak links with services. Outcome 3 rests on three outputs:

- SHG meetings are aligned with Anganwadi and VHSND calendars, and families receive help with documentation (OP3.1). This removes timing and paperwork barriers on the demand side.
- A community growth-tracking and referral pathway that includes follow-up after discharge (OP3.2) addresses the weak SAM continuum described in the problem analysis [S12].
- Joint training of Anganwadi Workers, ASHAs and SHG leaders, together with regular block convergence meetings (OP3.3), builds shared routines on the supply side. It also creates a forum where gaps in service supply can be raised and resolved.

Indicators and targets.

- The percentage of pregnant women with four or more ANC visits. The target is the baseline value plus 20 percentage points.
- The percentage of eligible pregnant and lactating women receiving THR regularly. The target is the baseline value plus 20 percentage points.
- The percentage of eligible women registered for PMMVY. The target is 80%.
- The percentage of identified SAM children who are referred and complete follow-up after discharge. The target is 80%.

Means of verification. These indicators will be verified through MCP cards, Anganwadi, PMMVY and MTC records, the referral register, and the baseline and endline surveys.

Outputs: the building blocks

The outputs are the direct deliverables of the activities, and each has a verifiable target.

Output	Target
OP1.1: Household nutri-kitchen gardens established	About 1,200 households trained and supplied; 80% of gardens functional (at least 5 crop types) in both the kharif and rabi seasons
OP1.2: Demonstration plots and seed banks functional	20 demonstration plots and 20 seed banks managed by Village Organisations
OP2.1: CNRP cadre trained and active	40 CNRPs trained; at least 36 active in any month
OP2.2: Counselling, cooking demonstrations and home visits delivered	440 cooking demonstrations; 80% of pregnant and lactating women receive the planned home visits
OP3.1: SHG meetings linked with Anganwadi and VHSND calendars	SHG participation recorded in VHSND in all 20 villages
OP3.2: Community growth-tracking and referral pathway functional	A supplementary growth-monitoring kit in use in all 20 villages; all identified SAM cases referred
OP3.3: Frontline workers trained and convergence meetings institutionalised	8 joint training batches; 8 block and 4 district convergence meetings
OP4: MEL delivered and handover completed	2 survey rounds; at least 4 learning products; 20 handover resolutions passed by Village Organisations

The target of at least 36 active CNRPs out of 40 builds in redundancy. Training two CNRPs per village protects coverage against drop-out.

OP4 serves every outcome. The tracking system allows the project to see participation and adjust its effort month by month. The baseline and endline surveys measure change. The handover resolutions formally transfer ownership to Village Organisations.

Assumptions linking the levels

The chain depends on assumptions that are stated openly here.

- Enough homestead land and water are available for household-scale gardens. The project manages this by sizing plots to available resources and seeking MGNREGA water works.
- Anganwadi and health services are available to meet the demand the project creates. The project manages this through service mapping, convergence meetings and separate tracking of service availability.
- CNRPs remain engaged throughout the project. The project manages this through selection by SHGs, endorsement by Village Organisations, training two CNRPs per village and recognition through the federation.

The next chapter describes the activities that deliver each output. It also explains how each activity produces its result.

Level	Result statement	Indicator	Baseline	Target	Means of verification
Impact	Contribution to improved nutritional status of mothers and children under five in target SHG households	% of children under five underweight; % of children under five stunted (contribution only, not attributed to the project)	To be established by baseline survey (state reference, NFHS-5: 39% underweight, 40% stunted)	A declining trend against the project baseline, reported as contribution	Baseline and endline anthropometric survey; Poshan Tracker and Anganwadi records
Outcome 1	Target households eat more diverse diets year-round	% of women (PW/LW) achieving Minimum Dietary Diversity for Women (MDD-W); % of households consuming garden produce in each season	To be established by baseline survey	MDD-W: baseline + 20 percentage points; garden produce consumed in at least 2 seasons by 70% of households	Baseline and endline survey (24-hour dietary recall); household tracking system

Outcome 2	Recommended maternal nutrition and IYCF practices adopted	% of children aged 6–23 months achieving Minimum Dietary Diversity; % of infants under 6 months exclusively breastfed; % of children aged 6–23 months eating iron-rich foods	To be established by baseline survey (state reference, NFHS-5: 22% of children 6–23 months ate iron-rich foods)	Child MDD: baseline + 15 percentage points; exclusive breastfeeding and iron-rich food intake show significant improvement on baseline	Baseline and endline survey; CNRP home-visit records
Outcome 3	Uptake of ICDS and health entitlements improved, and at-risk children are referred and followed up	% of pregnant women with 4+ ANC visits; % of eligible PW/LW receiving THR regularly; % of eligible women receiving PMMVY; % of identified SAM children referred who complete post-discharge follow-up	To be established by baseline survey	ANC 4+ and THR: baseline + 20 percentage points; PMMVY: 80% of eligible women registered; SAM follow-up completed for 80% of referred children	Baseline and endline survey; MCP cards; Anganwadi, PMMVY and MTC records; referral register
Output 1.1	Household nutri-kitchen gardens established	Number of households with a functional garden (at least 5 crop types) in each season	0 project-supported gardens (existing Didi Badi coverage to be mapped)	About 1,200 households trained and supplied; 80% functional in the monsoon (kharif) and winter (rabi) seasons	Training attendance records; distribution records; seasonal garden verification in the tracking system
Output 1.2	Demonstration plots and community seed banks functional	Number of demonstration plots and Village Organisation-managed seed banks functional	0	20 plots and 20 seed banks	Site visits; seed bank registers
Output 2.1	CNRP cadre trained and active	Number of CNRPs trained and active each month	0	40 trained; at least 36 active in any month	Training records; monthly activity reports
Output 2.2	Counselling, cooking demonstrations and home visits delivered	Number of cooking demonstrations; % of PW/LW receiving the planned home visits	0	440 demonstrations; 80% of PW/LW receive the planned home visits	Session registers; household tracking system

Output 3.1	SHG meetings linked with Anganwadi and VHSND calendars	Number of villages with SHG participation recorded in VHSND	To be established by baseline survey	20 villages	VHSND attendance records; SHG minutes
Output 3.2	Community growth-tracking and referral pathway functional	Number of villages with a supplementary growth-monitoring kit in use; number of referrals facilitated	0	20 villages; all identified SAM cases referred	Growth-tracking records reconciled with Anganwadi registers; referral register
Output 3.3	Frontline workers trained and convergence meetings institutionalised	Number of AWWs, ASHAs and SHG leaders trained; number of block and district convergence meetings held	0	8 joint training batches; 8 block and 4 district meetings	Training records; meeting minutes
Output 4	MEL delivered and handover completed	Baseline and endline surveys completed; learning products produced; seed banks and routines formally handed over	0	2 survey rounds; at least 4 learning products; 20 Village Organisation handover resolutions	Survey reports; publications; Village Organisation resolutions

Risk Management

The project's principal risks, their likelihood and impact, and how each is managed:

Risk	Likelihood	Impact	Mitigation
Duplication with or crowding by JSLPS Didi Badi / FNHW interventions or existing cadres in the target villages	Medium	High	Map coverage with the JSLPS Murhu Block Mission Management Unit in Q1. Choose villages or households to complement existing coverage. Formalise roles in writing, and redirect kit budget to counselling and convergence where gardens already exist.
Dry spells, erratic monsoon or limited water and homestead land reduce the year-round viability of gardens	Medium	High	Use drought-tolerant indigenous crops and millets, greywater use, mulching and staggered sowing. Take advice from KVK Khunti. Seek MGNREGA water works, and size plots to the water available.
Livestock damage to gardens	High	Medium	Use live fencing and local materials. Fence demonstration plots from project funds, and seek household fencing through MGNREGA convergence.
Weak Anganwadi or health service supply (THR irregularity, AWW or ASHA vacancies, VHSND not held) limits service uptake outcomes	Medium	High	Map service gaps in Q1. Raise supply issues at quarterly block and half-yearly district convergence meetings. Support VHSND through SHG participation. Track service availability separately from demand-side uptake.
No formal working relationship yet with the ICDS project office (CDPO), block health team or JSLPS	Medium	Medium	Seek letters of support or an MoU with the Murhu CDPO, the Medical Officer in charge of the CHC and the JSLPS Block Project Manager in the inception quarter. Use EcoKheti's existing SRLM relationship in Khunti.
CNRP drop-out because the honorarium is modest or domestic workload is heavy	Medium	Medium	Select CNRPs through SHGs with Village Organisation endorsement. Train two CNRPs per village for redundancy. Give recognition through the federation. Confirm the honorarium against the JSLPS CRP norm.
Remote villages and poor road connectivity or access conditions disrupt field presence and referral	Medium	Medium	Assess access and connectivity before selecting villages. Base each field facilitator in a cluster. Plan referrals using JSY/NHM transport, with a contingency fund.
Nutritional status changes are small or slow within 24 months, so the project appears to underperform	High	Medium	Set diets, practices and service uptake as the primary outcomes, as the evidence recommends. Present anthropometry as a contribution-level impact indicator only.

The indicative budget or target definition changes after donor engagement	Medium	Medium	The budget is modular (kits, CNRP numbers, surveys) and can be scaled. Confirm the target definition in the inception period.
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Sustainability

The activities described above are designed so that the project's gains outlast its 24 months. This chapter explains why that is credible. The central principle was set in the strategy. Because nothing is delivered through a parallel structure, nothing has to be dismantled or rescued when funding ends. Every capability the project builds sits inside an institution that already exists and will continue to exist.

Institutional ownership: Village Organisations and SHG federations

The seed banks will belong to Village Organisations from the start, and ownership will be formalised through handover resolutions in all 20 villages by Q8. Seed from the second year onwards comes from these banks, so the system will already have operated without project seed for a full year before handover. The project will therefore have tested the Village Organisations' capacity to manage seed during its lifetime, not merely assumed it.

The village nutrition action plans are developed and revised by the SHGs themselves. They give each group a nutrition agenda that it owns, and that agenda can continue on the regular SHG meeting cycle.

A cadre that stays: the CNRPs

The 40 CNRPs are SHG women living in their own villages. Their skills stay in the community whether or not the project continues. By the end of the project, they will be linked to SHG federations and, where possible, to JSLPS cadres. This gives them an institutional home and a possible route to continued recognition and remuneration within the state's community resource person system.

Confirming the CNRP honorarium against the JSLPS community resource person norm during implementation serves this purpose, because it keeps the role compatible with public cadres. The evidence that the women's group model has been delivered successfully through ASHAs [\[S13\]](#)[\[S14\]](#)[\[S15\]](#) shows that counselling of this kind can be sustained through government platforms.

Embedded routines with public systems

Convergence creates much of the lasting value. By the end of the project, the following routines will be in place:

- SHG meetings aligned with Anganwadi and VHSND calendars;
- SHG participation in VHSND;
- jointly trained Anganwadi Workers, ASHAs and SHG leaders;
- quarterly block convergence meetings recorded in official minutes.

These are routines within the ICDS, NHM and JSLPS systems, not project activities. Growth tracking is reconciled with Anganwadi registers throughout, so it strengthens the Anganwadi Worker's own records rather than creating a separate data stream. The referral and follow-up pathway links Anganwadi SAM detection to the MTC, and the Anganwadi Worker and the community can continue to use it.

Lasting assets and household benefits

Households keep lasting assets from the project. Fruit saplings, including moringa, papaya and lemon, will keep producing for years. The garden skills learned through training and demonstration remain with the households. Household fencing and water structures, sought through MGNREGA, belong to the families.

The public entitlements the project helps families access, such as THR, PMMVY, JSY, immunisation and antenatal care, continue to flow without any project cost. Women who have learned how to prepare documents and claim these benefits keep that capability for later pregnancies and for younger children.

Evidence for continuation and scale-up

The baseline and endline surveys, the tracking data and at least four learning products will document what worked in Murhu. Quarterly reviews with SHG federations and block officials mean that the people who will carry the work forward see these results as they emerge. Because the design avoids duplicating Didi Badi Yojana and aligns with Poshan Vatika, Poshan 2.0 and NHM, the approach can be extended by JSLPS or the district administration within existing programmes.

Honest limits

Sustainability depends on factors the project can influence but not control. These include the regularity of Anganwadi and health service supply, continued federation support for CNRPs, and the rainfall and water availability that gardens need. The project reduces these risks through service mapping, climate-resilient garden design, two CNRPs per village and formal handover. It does not claim that they disappear.

Nutritional status improves more slowly than diets and practices ^[S18]. The fuller benefit of better feeding in the first 1,000 days will appear in children's growth after the project cycle. This is a further reason why the routines that sustain these practices must remain after the project ends.

The golden thread therefore runs to the end. Undernutrition in Murhu arises from narrow diets, poor feeding practice and unclaimed entitlements, rooted in poverty, remoteness and the literacy gap. The project addresses each cause through women's own institutions and the State's own platforms. When the project closes, the seed banks, the counselling cadre and the convergence routines remain with those institutions and platforms.

Budget

Item	Unit	Unit cost	Qty	Total	Contributes to	Source
A. Personnel & Salaries						
Project Coordinator (1 FTE)	person-month	40,000	24	9,60,000	Overall management, block and district convergence, reporting	JSLPS block-level professional band Rs 25,000–40,000/month (GoJ Rural Development Department notification, 2019). Upper end used; escalation not applied.
Nutrition and Health Convergence Officer (1 FTE)	person-month	32,000	24	7,68,000	Counselling content, ICDS and health linkages, referral systems	JSLPS block-level band Rs 25,000–40,000 (2019). Middle of band, below the Project Coordinator.
Livelihoods / Kitchen Garden Specialist (50% FTE)	person-month (pro-rata)	18,000	24	4,32,000	Garden design, crop planning, climate-resilient practices, household training	JSLPS band (2019), 50% of a Rs 36,000 FTE, within the pro-rata range Rs 12,500–20,000
MEL and Data Officer (50% FTE)	person-month (pro-rata)	14,000	24	3,36,000	Tracking system, data quality, progress reporting	JSLPS band (2019), 50% of a Rs 28,000 FTE, lower end as advised
Field Facilitators / Cluster Coordinators (4, one per 5-village cluster)	person-month	18,000	96	17,28,000	Village implementation, SHG facilitation, CNRP supervision	Estimate based on typical NGO field facilitator pay in rural Jharkhand (graduate, two-wheeler travel). No authoritative norm found.
Community Nutrition Resource Persons (40 SHG women, 2 per village, months 3–24)	honorarium per person-month	3,500	880	30,80,000	Group counselling, cooking demonstrations, home visits, household tracking	Estimate set above the NHM ASHA-facilitator floor of about Rs 3,000/month (Rs 150/day × 20 days), which is only a weak proxy
Subtotal — Personnel & Salaries				73,04,000		
B. Programme / Activity Costs						

Kitchen garden seed kits (vegetables, greens, millets, pulses)	per household kit per season	350	2,400	8,40,000	Garden establishment in about 1,200 households for 2 seasons in Year 1; Year-2 seed from seed banks	Estimate for a multi-crop local kit. The Odisha minikit rate of Rs 75 (2009-10) is out-of-state, dated and seeds only, so it is used for order of magnitude only.
Fruit sapling set (5 saplings, e.g. moringa, papaya, lemon)	per household (one-time)	250	1,200	3,00,000	Perennial micronutrient sources	Estimate from local nursery pricing. No Jharkhand norm found.
Village demonstration plots and community seed banks	per village	25,000	20	5,00,000	Hands-on learning and continued seed availability	Estimate for plot inputs, seed storage containers and registers. No official per-village norm found.
Low-cost water management and fencing for demonstration plots	per demonstration plot	15,000	20	3,00,000	Year-round viability and protection from livestock; household fencing sought via MGNREGA	Estimate for live or bamboo fencing, greywater channel and drum storage. No official norm found.
Household kitchen garden training (1-day village batches of 30)	per training batch	12,000	40	4,80,000	Skills for garden establishment and maintenance	ATMA cafeteria norm Rs 400 per trainee per day (MANAGE; Bilaspur district site) × 30 × 1 day, scaled to a non-residential session
CNRP initial residential training (3 batches × 2 modules, 2 days each)	per 2-day batch of about 15	50,000	6	3,00,000	Counselling capacity of the community cadre	BPKP guidelines (2021): Rs 50,000 per 2-day training of 15 CRPs by external resource persons
CNRP non-residential refreshers (6 days × 40)	per participant-day	400	240	96,000	Ongoing skill reinforcement	ATMA cafeteria rate Rs 400 per trainee per day (MANAGE cost norms)
Joint AWW, ASHA and SHG leader trainings (8 batches of about 30, 1 day)	per training batch	13,000	8	1,04,000	Shared convergence routines with frontline workers	Build-up: 2 resource persons × Rs 500 (NHM BRIDGE norm) plus about Rs 400 per participant for TA/DA and logistics (ATMA proxy). State TA/DA norm not found.

Cooking demonstrations and nutrition session materials	per session	500	440	2,20,000	Behaviour change on dietary diversity and IYCF	Estimate for local food ingredients and fuel. The POSHAN community-event rate was seen only in an unofficial source, so it is not cited.
Pictorial counselling flipbook kits in local language (40 CNRP + 20 Anganwadi centre)	per set	2,500	60	1,50,000	Consistent messages for low-literacy audiences	Estimate from typical design and print costs for laminated flipbooks. No per-set norm found.
Household pictorial take-home cards	per household	50	1,200	60,000	Reinforcing messages at home	Estimate from local print pricing
Supplementary growth-monitoring kit (scale, MUAC tapes, infantometer)	per set per village	6,000	20	1,20,000	Community growth tracking alongside the AWW	Estimate. MWCD device specifications exist, but no unit cost was found.
Referral and follow-up support (transport or escort contingency)	per referral case	1,500	60	90,000	Access of at-risk children and mothers to CHC and MTC where government transport is unavailable	Estimate of local travel and escort cost. The case count is a planning figure, not a prevalence estimate.
Block-level convergence meetings (quarterly)	per meeting	750	8	6,000	Institutionalising coordination with ICDS and health	NHM ASHA support budget provisions: Rs 9,000 per year for monthly block meeting refreshments (about Rs 750 per meeting)
District-level convergence meetings (half-yearly)	per meeting	5,000	4	20,000	District buy-in from DSWO, Civil Surgeon and JSLPS	Estimate for venue, refreshments and materials. The NHM norm covers block refreshments only.
Exposure visits within Jharkhand (60 participants × 2 days × 2 visits)	per participant per day	400	240	96,000	Peer learning for SHG leaders and CNRPs	ATMA cafeteria exposure visit norm, within-state rate Rs 400 per participant per day (Bilaspur district site)

Baseline and endline surveys	per survey round	3,50,000	2	7,00,000	Measuring change in outcome indicators, including anthropometry	Estimate for a sample survey of about 500 households by a regional agency. No official norm found.
Mobile devices for household tracking (4 field facilitators + 20 lead CNRPs)	per device	10,000	24	2,40,000	Tracking mothers, children and gardens (open-source data collection tool)	Estimate for an entry-level Android smartphone. The MWCD POSHAN smartphone figure was not found.
Mobile data for tracking devices	per device-month	150	576	86,400	Data upload and review	Estimate from prepaid data plan pricing
Documentation and learning products	lump sum	1,50,000	1	1,50,000	Case studies, learning briefs, dissemination	Estimate for writing, design, printing and a dissemination event
Staff travel and local conveyance (6 field-based staff)	per person-month	3,000	144	4,32,000	Field presence across 20 villages	Estimate for two-wheeler fuel and reimbursement in rural Khunti. No norm found.
Subtotal — Programme / Activity Costs				52,90,400		
C. Administrative / Overheads						
Field office running costs, Murhu (rent, utilities, internet, stationery)	per month	20,000	24	4,80,000	Operational base in or near Murhu	Estimate for small block-level office costs in rural Jharkhand. Local quote needed.
Administrative and overhead costs (finance, compliance, statutory audit, HQ management)	7% of direct costs (Rs 1,30,74,400)	9,15,208	1	9,15,208	Organisational management, finance, compliance and audit	Estimated rate set to fit the ceiling. No scheme norm found; the donor's permitted rate will apply.
Subtotal — Administrative / Overheads				13,95,208		
Grand Total				1,39,89,608		

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Poshan Saathi: SHG-led Kitchen Gardens, Nutrition Counselling and ICDS Convergence for Mothers and Young Children in Murhu, Khunti

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